

This Patient Group Direction (PGD) is intended only for registered healthcare professionals who have been named and authorised by their organisation to practice under it.

Patient Group Direction

for the administration of Sildenafil or Tadalafil for the treatment of Erectile Dysfunction

Summary of NICE / NICE CKS Guidelines for Erectile Dysfunction

Overview

- **Erectile dysfunction (ED)** is defined as the persistent inability to attain or maintain an erection sufficient for satisfactory sexual performance.
- It is common, affecting approximately 50% of men aged 40–70 years to some degree.
- Causes may be organic (vascular, neurological, hormonal, drug-related), psychogenic, or mixed.
- ED may be an early marker of cardiovascular disease and should prompt assessment of cardiovascular risk factors.

Assessment

- **Take a full medical history** including cardiovascular risk factors, diabetes, neurological conditions, medications, and psychological health.
- Assess lifestyle factors: smoking, alcohol, obesity, physical inactivity.
- Enquire about the nature, onset, and duration of ED (gradual onset suggests organic cause; sudden onset suggests psychogenic).
- Perform cardiovascular risk stratification before initiating treatment.
- Check blood pressure, fasting glucose or HbA1c, lipid profile, and early morning testosterone if clinically indicated.

Red Flags – Consider Referral

- Pelvic or perineal trauma
- Penile anatomical abnormality (e.g. Peyronie's disease)
- History of priapism or sickle cell disease
- Unexplained hypogonadism or endocrine disorder
- Complex psychiatric or psychosexual issues
- Failure to respond to two different PDE5 inhibitors at maximum dose

Management – First-Line Treatment

- **First-line pharmacological treatment:** PDE5 inhibitors (phosphodiesterase type 5 inhibitors).
- **Sildenafil** is the first-line choice. Start at 50 mg taken approximately 1 hour before sexual activity. Adjust to 25 mg or 100 mg based on efficacy and tolerability.
- **Tadalafil** is an alternative. On-demand dosing: start at 10 mg, taken at least 30 minutes before activity, adjustable to 20 mg. Daily dosing: 2.5–5 mg once daily.
- Advise the patient to trial the medication on at least 6–8 separate occasions with sexual stimulation before concluding treatment failure.

- Do not prescribe PDE5 inhibitors to patients taking nitrates (absolute contraindication) or those for whom sexual activity is inadvisable due to cardiovascular status.

Lifestyle Advice

- Smoking cessation
- Weight loss if overweight or obese
- Increased physical activity
- Reduce alcohol intake
- Address psychosexual factors where appropriate

When to Refer or Escalate

- Failure to respond to two different PDE5 inhibitors at maximum dose after adequate trial (6–8 attempts each)
- Suspected hypogonadism (low testosterone) requiring specialist assessment
- Complex psychological, relationship, or psychosexual issues
- Significant cardiovascular risk factors requiring cardiology input before initiating treatment
- Penile abnormalities, priapism history, or sickle cell disease

Patient Group Direction

for the supply/administration of

Sildenafil

for the treatment of

Erectile Dysfunction

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	• Pharmacist registered and practicing with the GPhC • Pharmacy technician registered and practicing with the GPhC
Training and assessment	Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. • All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines • All users must previously have used PGDs to supply medication • Must work in compliance with the SOPs of their own employer • All users must have appropriate indemnity in place to cover this service
Further training and responsibilities	• All users must be up to date with CPD requirements and appraisal • All users must be up to date with MHRA and safety alerts with regards to medical products • All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005.

PGD Indication	Sildenafil is indicated for the treatment of erectile dysfunction in adult males aged 18 years and over, defined as the inability to achieve or maintain a penile erection sufficient for satisfactory sexual performance.
Inclusion criteria	- Adult males aged 18 years and over. - Clinical presentation consistent with erectile dysfunction. - Informed consent obtained. - No contraindications to sildenafil. - Cardiovascular risk assessment completed and deemed low risk for sexual activity.
Exclusion criteria	- Known hypersensitivity to sildenafil or any excipient.

	- Concurrent use of organic nitrates in any form (e.g. glyceryl trinitrate, isosorbide mononitrate/dinitrate) or nitric oxide donors (e.g. amyl nitrite) – absolute contraindication due to risk of severe hypotension. - Concurrent use of riociguat (a guanylate cyclase stimulator). - Severe hepatic impairment. - Recent stroke or myocardial infarction (within the last 6 months). - Hypotension (blood pressure <90/50 mmHg). - Uncontrolled hypertension (blood pressure >170/100 mmHg). - Known hereditary degenerative retinal disorders (e.g. retinitis pigmentosa). - Conditions where sexual activity is inadvisable (e.g. unstable angina, severe heart failure NYHA class IV, uncontrolled arrhythmias). - Previous episode of non-arteritic anterior ischaemic optic neuropathy (NAION).
Cautions	- Anatomical deformation of the penis (e.g. angulation, cavernosal fibrosis, Peyronie's disease). - Conditions predisposing to priapism (sickle cell anaemia, multiple myeloma, leukaemia). - Active peptic ulceration or bleeding disorders. - Concurrent use of alpha-blockers: ensure patient is stable on alpha-blocker therapy before initiating sildenafil; start with 25 mg. - Concurrent use of CYP3A4 inhibitors (e.g. erythromycin, ketoconazole, itraconazole, ritonavir): consider starting dose of 25 mg. - Mild to moderate hepatic impairment: consider starting dose of 25 mg. - Severe renal impairment (eGFR <30 mL/min): consider starting dose of 25 mg. - Patients with cardiovascular disease should have risk assessed before treatment initiation. - Advise the patient that sudden visual loss may indicate NAION; discontinue immediately and seek urgent medical attention.
Actions if patient is excluded or declines treatment	Advise on alternative treatment options and how these can be accessed. Document any advice given and the decision reached. Inform or refer to the GP as appropriate.

Details of the medicine

Name, form and strength of medicine	Sildenafil 25 mg, 50 mg, and 100 mg film-coated tablets
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Legal category	POM
Storage	Store below 30°C in the original packaging.
Route/method of administration	Oral. Taken approximately 1 hour before sexual activity. Can be taken from 30 minutes to 4 hours before sexual activity. Sexual stimulation is required for the medication to be effective.
Dose and frequency	Starting dose: 50 mg taken as needed, approximately 1 hour before sexual activity. The dose may be increased to a maximum of 100 mg or decreased to 25 mg depending on efficacy and tolerability. Maximum one dose per 24-hour period. Efficacy may be reduced if taken after a high-fat meal. Consider 25 mg starting dose in patients aged over 65, patients with hepatic or severe renal impairment, and those taking CYP3A4 inhibitors or alpha-blockers.
Quantity to be administered and/or supplied	Up to 8 tablets per supply (one month's treatment based on twice-weekly use). A clinical decision should be made on the quantity based on individual need.
Maximum or minimum treatment period	No maximum treatment period. Review effectiveness and cardiovascular risk at least annually. The patient should be reassessed if no improvement after 6–8 attempts at the maximum tolerated dose.
Adverse effects	Very common ($\geq 1/10$): Headache. Common ($\geq 1/100$ to $< 1/10$): Flushing, dyspepsia, nasal congestion, dizziness, visual disturbance (including colour tinge, increased brightness, blurred vision), nausea. Uncommon: Vomiting, rash, epistaxis, palpitations, tachycardia, drowsiness, eye pain, tinnitus. Rare but serious: Priapism (erection lasting > 4 hours – requires immediate medical attention), non-arteritic anterior ischaemic optic neuropathy (NAION), sudden sensorineural hearing loss. Refer to SmPC for full list of adverse effects.

Yellow card reporting	Report suspected adverse effects via https://yellowcard.mhra.gov.uk and inform the GP as appropriate.
Records to be kept	Record the following information: • that valid informed consent was given • name of individual, address, date of birth and GP • name of healthcare practitioner • name and brand of medication • date of supply dose, form and route quantity supplied

	• advice given, including advice given if excluded or declines treatment • details of any adverse drug reactions and actions taken supplied via PGD Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. A record of all individuals receiving treatment under this PGD should also be kept for audit purposes as follows: • Adults aged 18 years and over - keep records for 8 years • Children aged under 18 years - keep records until the 25th birthday (if the patient was 17 years when treatment was finished, keep records until the 26th birthday)
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Patient information

Written information to be given to patient or carer	Supply the patient information leaflet (PIL) provided with the medication.
Follow-up advice to be given to patient or carer	To seek medical advice if symptoms worsen or they experience any adverse effects. The medication does not cause automatic erections; sexual stimulation is required for it to work. Seek immediate medical attention if an erection lasts longer than 4 hours (priapism). Seek urgent medical attention if there is a sudden loss of vision in one or both eyes, or a sudden decrease or loss of hearing. Avoid grapefruit juice while taking the medication as it may increase side effects. Advise the patient that the medication does not protect against sexually transmitted infections. If the patient is not already under regular GP review, recommend a cardiovascular risk assessment.

Key references

• NICE Medicines Practice Guideline 2 (MPG2): Patient Group Directions. Published March 2017 https://www.nice.org.uk/guidance/mg2 • NICE MPG2 Patient group directions: competency framework for health professionals using patient group directions. Updated March 2017 https://www.nice.org.uk/guidance/mpg2/resources • NICE CKS Guidance: Erectile Dysfunction • British National Formulary (BNF) Key information on the selection, prescribing, dispensing and administration of medicines • Summary of Product Characteristics for Sildenafil and Tadalafil
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Agreement to practise by the Registered Healthcare Professional

By signing this PGD you are confirming that you agree to its contents and that you will work within it. PGDs do not remove inherent professional obligations or accountability. It is the responsibility of each healthcare professional to practise only within the bounds of their own competence and professional code of conduct.

Name and address of pharmacy premises / clinic premises to which this PGD relates			
Name of healthcare professional	Job title	Registration number	Signature
Valid from date	Expiry date		
1/11/25	31/10/26		

PGD adoption and authorisation by the employer/clinical lead

The employer has ensured that the person using this PGD, has the knowledge and skills to safely provide the service which will encompass the supply or administration of a medication or medicine.

This section must be signed by the superintendent or clinical lead responsible for this service.

Name of superintendent / clinical lead	Job title & name of organisation	Signature	Date
Name of professional group signatory	Job title & name of organisation	Signature	Date

Signed on behalf of Get Real Health

Name	Qualification	Signature	Date
Nitin Shori	Medical Doctor GMC: 6047293		2/11/25
Chris Pilkington	Pharmacist GPhC: 2046322		2/11/25

Change history

Version number	Change details	Date
001	Development & issue of new PGD	1st November 2025

Patient Group Direction

for the supply/administration of

Tadalafil

for the treatment of

Erectile Dysfunction

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	• Pharmacist registered and practicing with the GPhC • Pharmacy technician registered and practicing with the GPhC
Training and assessment	Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. • All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines • All users must previously have used PGDs to supply medication • Must work in compliance with the SOPs of their own employer • All users must have appropriate indemnity in place to cover this service
Further training and responsibilities	• All users must be up to date with CPD requirements and appraisal • All users must be up to date with MHRA and safety alerts with regards to medical products • All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005.

PGD Indication	Tadalafil is indicated for the treatment of erectile dysfunction in adult males aged 18 years and over. Available as on-demand dosing or once-daily dosing for men who anticipate frequent sexual activity (at least twice per week).
Inclusion criteria	- Adult males aged 18 years and over. - Clinical presentation consistent with erectile dysfunction. - Informed consent obtained. - No contraindications to tadalafil. - Cardiovascular risk assessment completed and deemed low risk for sexual activity.
Exclusion criteria	- Known hypersensitivity to tadalafil or any excipient.

	- Concurrent use of organic nitrates in any form – absolute contraindication. - Concurrent use of riociguat (guanylate cyclase stimulator). - Recent stroke or myocardial infarction (within the last 6 months). - Hypotension (blood pressure <90/50 mmHg) or uncontrolled hypertension (>170/100 mmHg). - Severe hepatic impairment (Child-Pugh Class C). - Severe renal impairment (creatinine clearance <30 mL/min) for once-daily dosing. - Conditions where sexual activity is inadvisable (e.g. unstable angina, severe heart failure, uncontrolled arrhythmias). - Known hereditary degenerative retinal disorders. - Previous episode of NAION.
Cautions	- Anatomical deformation of the penis (angulation, cavernosal fibrosis, Peyronie's disease). - Conditions predisposing to priapism (sickle cell anaemia, multiple myeloma, leukaemia). - Active peptic ulceration or bleeding disorders. - Concurrent use of alpha-blockers: patient must be stable on alpha-blocker before initiating tadalafil. - Concurrent use of potent CYP3A4 inhibitors (e.g. ketoconazole, ritonavir): on-demand dose should not exceed 10 mg in a 72-hour period. - Mild to moderate hepatic impairment (Child-Pugh A or B): on-demand dose should not exceed 10 mg. - Moderate renal impairment (creatinine clearance 30–50 mL/min): for once-daily dosing, start at 2.5 mg. - Tadalafil has a prolonged half-life (~17.5 hours); adverse effects may persist for longer than sildenafil. - Patients with cardiovascular disease should have risk assessed before treatment.
Actions if patient is excluded or declines treatment	Advise on alternative treatment options and how these can be accessed. Document any advice given and the decision reached. Inform or refer to the GP as appropriate.

Details of the medicine

Name, form and strength of medicine	Tadalafil 2.5 mg, 5 mg, 10 mg, and 20 mg film-coated tablets
Legal category	POM

Storage	Store below 30°C in the original packaging. Protect from moisture.
Route/method of administration	Oral. On-demand dosing: taken at least 30 minutes before sexual activity. Can be effective for up to 36 hours. Once-daily dosing: taken at approximately the same time each day, irrespective of timing of sexual activity. Sexual stimulation is required.
Dose and frequency	On-demand dosing: Starting dose 10 mg taken at least 30 minutes before sexual activity. May be increased to 20 mg or decreased to 5 mg based on efficacy and tolerability. Maximum one dose per 24 hours. Once-daily dosing: 2.5 mg once daily at approximately the same time, which may be increased to 5 mg once daily based on tolerability. Suitable for men anticipating frequent sexual activity (at least twice per week). Absorption is not affected by food. With potent CYP3A4 inhibitors: on-demand use should not exceed 10 mg in a 72-hour period. Mild to moderate hepatic impairment: on-demand dose should not exceed 10 mg.
Quantity to be administered and/or supplied	On-demand: Up to 8 tablets per supply. Once-daily: up to 28 tablets per supply (one month). A clinical decision should be made on the quantity based on individual need.
Maximum or minimum treatment period	No maximum treatment period. Review effectiveness and cardiovascular risk at least annually. For once-daily dosing, periodically reassess whether continued daily use is appropriate.
Adverse effects	Very common ($\geq 1/10$): Headache. Common ($\geq 1/100$ to $< 1/10$): Back pain, myalgia, pain in extremity, flushing, nasal congestion, dyspepsia, gastro-oesophageal reflux. Uncommon: Dizziness, abdominal pain, hyperhidrosis, rash, tachycardia, palpitations. Rare but serious: Priapism (erection lasting > 4 hours – requires immediate medical attention), NAION, sudden sensorineural hearing loss. Note: Due to the prolonged half-life of tadalafil, adverse effects may persist for a longer duration compared to sildenafil. Refer to SmPC for full list of adverse effects.

Yellow card reporting	Report suspected adverse effects via https://yellowcard.mhra.gov.uk and inform the GP as appropriate.
Records to be kept	Record the following information: - that valid informed consent was given - name of individual, address, date of birth and GP

	• name of healthcare practitioner • name and brand of medication • date of supply dose, form and route quantity supplied • advice given, including advice given if excluded or declines treatment • details of any adverse drug reactions and actions taken supplied via PGD Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. A record of all individuals receiving treatment under this PGD should also be kept for audit purposes as follows: • Adults aged 18 years and over - keep records for 8 years • Children aged under 18 years - keep records until the 25th birthday (if the patient was 17 years when treatment was finished, keep records until the 26th birthday)
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Name of professional group signatory	Job title & name of organisation	Signature	Date
	(GPhC: 2046322)		

Signed on behalf of Get Real Health

Name	Qualification	Signature	Date
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